

CLINICAL PRACTICE

AI Scribe & AI Tools: Consent, Vendor Vetting & Practice Policy

The paperwork behind using AI in a clinical practice: a patient consent form, a vendor checklist, a one-page policy and a note-review attestation.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Consent + checklist + policy

VERSION

1.0 · Sept 2026

TIME TO ADAPT

45 minutes

WHAT IT IS

A complete kit for using AI tools in your practice lawfully and transparently: a patient consent form, a vendor vetting checklist, a one-page AI policy, and an attestation line for notes an AI has drafted.

USE IT WHEN

You use, or are considering, an AI scribe, transcription tool or drafting assistant, or you are asked about AI by patients, supervisees or your consultation group.

CUSTOMIZE FIRST

Your state's AI, recording and telehealth rules, your vendor's actual data terms, and whether you record audio or use transcription only.

What has changed

AI note-taking tools are now among the topics clinicians search for most, and states have begun to regulate them. Compliance summaries published in mid-2026 report that several states now require written, revocable patient consent before AI records or transcribes a psychotherapy session, including Illinois, Maine, Colorado and Rhode Island, with disclosure or note-review duties in others (for example Texas, Louisiana and Nevada). Separately, roughly a dozen states require all parties to consent to recording. Class actions have been filed in California over ambient scribes under wiretap and privacy laws. Laws are moving quickly, so confirm each item below with your board or an attorney.

THREE RULES THAT HOLD ALMOST EVERYWHERE

1) A vendor that touches patient information needs a signed Business Associate Agreement (BAA) before first use. A general consumer chatbot without a BAA is not a HIPAA-compliant place for identifiable patient information. 2) A BAA does not replace patient consent or your clinical responsibility. 3) You must review and remain accountable for every AI-drafted note.

Part A: Do I need consent? A quick decision flow

1. Does the tool hear, record or transcribe the session, or receive session content? If yes, obtain written consent, and treat the audio and transcript as protected health information.

2. Is any participant in an all-party-consent state (yours or the patient's)? If yes, every participant must agree before recording begins.
3. Do you practice in, or treat patients located in, a state with an AI-in-therapy law? If yes, follow its consent, review and disclosure rules. Consent typically cannot be buried in general terms of use.
4. Is the tool used only for administration (scheduling, billing) with no clinical content? A lighter disclosure may suffice, but keep the BAA.
5. Do not offer AI-delivered therapy. Several states restrict it.

Part B: Patient consent for AI documentation tools

Consent to the use of an AI documentation assistant.

[Clinician name] uses [Tool name, vendor] to help write clinical notes. This is what it does: [it listens to our session and drafts a note / it transcribes and summarizes / it helps me organize notes I write]. I review and finalize every note. The tool does not make clinical decisions and is not a replacement for my judgment.

Recording and storage. Audio is [not recorded / recorded and deleted within (number) days]. Transcripts and drafts are [stored for (period) / deleted after I finalize the note]. Data is encrypted and protected under a Business Associate Agreement with the vendor. The vendor [does not / may not] use my information to train its models.

Who can see it. Only [Clinician] and authorized staff. The vendor can access data only as needed to provide the service and as the BAA allows.

Your choice. Agreeing is voluntary. If you decline or later withdraw consent, I will not use the tool for your sessions and your care will not be affected. You can withdraw at any time by telling me. If you withdraw, I will stop using it for future sessions.

Who else is present. If anyone else is with you during a session, I will ask for their consent as well.

I have had the chance to ask questions.

Patient name

Date

☐ I consent to the use of the AI tool as described above.

☐ I do not consent. Please do not use it for my sessions.

Patient signature

Date

Clinician signature

Date

Part C: Vendor vetting checklist

Complete before you sign, and repeat annually. Keep the answers with your Security Risk Analysis (PA-18).

Question	What good looks like	Vendor answer
Will the vendor sign a BAA before any use?	Yes, signed, and on your plan tier (some vendors sign only on business tiers).	
Do they use your data to train or improve models?	No, contractually prohibited. No “de-identified” loophole without your consent.	
Is audio recorded or stored? For how long?	Configurable. Audio deleted promptly or never stored.	
Where is data stored and who are the subprocessors?	Named, listed in the contract, in the United States or a location you accept.	
Encryption in transit and at rest? MFA? Role-based access?	Yes to all. Audit logs available.	
Independent security attestation?	SOC 2 Type II, HITRUST or equivalent, available on request.	
Breach notification terms?	Notice to you within days, not weeks, with cooperation clauses.	
Can you delete your data on demand and at contract end?	Yes, with written confirmation.	
How are psychotherapy notes handled?	Kept separate from the record or excluded by design.	
Does the tool work in states with all-party consent or AI laws where you practice?	Built-in consent workflow, easy opt-out.	
Is a consumer or free tier being used?	No. Enterprise or clinical tier only.	

Part D: One-page practice AI policy

[Practice name] AI Use Policy. Effective **[date]**. Reviewed **[annually]**.

Permitted. Use of vendors under a signed BAA to draft, transcribe or summarize with patient consent, and administrative tools without clinical content.

Prohibited. Entering identifiable patient information into any tool without a BAA. Using AI to make or finalize a diagnosis or treatment decision without clinician judgment. Using AI to conduct therapy or to communicate therapeutic content to patients on the clinician’s behalf.

Consent. Written, revocable consent for any tool that records, transcribes or receives session content, kept in the record. No penalty for declining.

Review. Every AI-drafted note is reviewed and corrected by the clinician before it is signed. Errors, hallucinations and omissions are corrected in the record.

Consultation and supervision. No AI note-takers or transcription in consultation groups or supervision.

Incidents. Suspected misuse or a vendor incident is reported to **[privacy officer]** and handled under the breach response plan (PA-18).

Review. Reviewed each year and whenever laws or vendors change.

Part E: Note attestation

Add a short line to any note that an AI drafted:

This note was drafted with [Tool] and reviewed, edited and approved by me on [date]. I am responsible for its accuracy. [Clinician signature and credential]

How to raise it with a patient

"I use a secure tool that helps me write my notes so I can pay more attention to you during the session. It [listens and drafts / transcribes], and I always review the note. I will give you a short form that explains how it works. You are free to say no, and it will not change your care. What questions do you have?"

PATIENTS WITH SPECIAL PROTECTIONS

Take extra care with minors, patients in legal proceedings, and any patient who has said their sessions are highly sensitive. Consider not using ambient recording at all in those cases.

IN PSYALLIANCE

If you use AI tools in your practice, note your policy in your informed consent and keep vendor BAAs and this checklist in your personal library. The Credentials area can hold your insurance panels and compliance dates.

Sources and further reading

- HHS, HIPAA Privacy and Security Rules and business associates ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- State AI-in-psychotherapy and recording-consent laws reported by compliance summaries (Illinois WOPR Act, Maine LD 2082, Colorado HB 26-1195, Rhode Island 2026 laws). Verify current text with your board.
- APA Practitioner Pulse Survey 2024, on clinician use of AI tools ([apa.org](https://www.apa.org)).

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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